The ACA's Second Act

While the majority of Americans know of the Affordable Care Act (ACA), also known as Obamacare, few know of the enhanced Premium Tax Credit rules associated with it and their recent changes. The ACA, itself, is a major health-care law passed in 2010 under Obama's presidency, thus aptly named, designed to make health insurance more available for the everyday American. It allowed young adults to stay on a parent's health plan until the age of 26, and prevented insurers from denying coverage or charging more simply because someone had a pre-existing condition. But it is important to note that the ACA did not create one government-run health insurance plan for everyone, but it largely worked through numerous private insurance companies, aided by government rules and financial assistance.

Before the ACA, the US health-insurance system had several major gaps. Namely, insurers in the private sector could make coverage difficult for individuals with pre-existing conditions, and many people who didn't get insurance through an employer had trouble affording coverage. The ACA was designed as a broad reform of that existing system, and as former President Barack Obama signed the Patient Protection and Affordable Care Act into law in 2010, and later when Congress passed the Health Care and Education Reconciliation Act modifying parts of the original law, thus birthed what we now generally refer to as the Affordable Care Act.

The Financial Safety Net

Firstly, Premium Tax Credits (PTCs) are a sort of discount on your monthly health-insurance premium, essentially financial assistance from the federal government that helps eligible individuals pay for health insurance they buy through an ACA Marketplace.

Secondly, when Congress passed the Affordable Care Act in 2010, it simultaneously created the Premium Tax Credit through Section 36B of the Internal Revenue Code. The credit was primarily designed to help low income individuals and families pay the premiums for insurance based through the new ACA Marketplace.

But although Congress created the credit in 2010, it didn't become available until 2014 when the ACA's Marketplace insurance system began operating at the same time. And that's when eligible people could actually start receiving the Premium Tax Credit. The credit was refundable, so one could receive the benefit even if they owed little or no federal income tax; it was also advanceable, meaning people didn't necessarily have to wait until they filed their taxes. All this means is that if an individual's insurance costs $500/month, then they wouldn't have to wait until tax season for assistance as the government would provide an estimated $300 credit which the insurer would then receive, leaving only $200 to be paid out-of-pocket.

From Temporary Relief to Enhanced Subsidies

Then came COVID-19. And the post-Covid era, specifically, the year 2021 where subsidies dramatically expanded. In 2021 a few key things occurred:

The American Rescue Plan Act (ARPA), a $1.9 trillion economic stimulus package designed to respond to COVID-19 public health and economic crises at the time, was signed into law in March 2021. To make healthcare more affordable during the pandemic ARPA temporarily enhanced ACA Marketplace subsidies, allowing individuals with household incomes over 400% of the federal poverty line (FPL) to qualify for financial assistance if the cost of their insurance plan exceeded 8.5% of their income. It lowered the percentage of income that all eligible households were expected to contribute towards their premiums. And it created a benchmark coverage entirely free for lower-income individuals.

These enhanced subsidies were originally temporary and were scheduled to expire after 2022. However, Congress extended the ACA Premium Tax Credits through 2025 under the Inflation Reduction Act (IRA) of 2022 primarily to stabilize the health insurance market as the country emerged from the pandemic, and to prevent a shock for millions of Americans when their monthly premiums would double or triple virtually overnight. Health insurers warned that if the subsidies expired earlier, healthier individuals would be the first to cancel their plans due to higher cost. This would leave a smaller, older, and sicker "risk pool" in the Marketplaces, forcing insurers to drastically raise premiums for everyone else to cover the costs. Extending the subsidies gave insurers the certainty they needed to keep premium rates stable.

The Debate Over What Comes Next

Throughout 2025, there were major political debates over whether Congress should continue the enhanced subsidies or not. Congress ultimately did not extend them.

The 2025 federal reconciliation law did not renew the enhanced ACA Premium Tax Credits, and coming into 2026, the regular premium-tax-credit rules returned, including the traditional income ceiling of 400% of the federal poverty level. For many people buying insurance through the ACA Marketplace, insurance became visibly more expensive.

And this has already had measurable effects. KFF's analysis of 2026 enrollment found that while average monthly premium payments increased from $113 in 2025 to $178 in 2026, a 58% increase, the Marketplace enrollment simultaneously declined (McGough et al, 2026).

Three Paths Forward

So what happens to the ACA now?

The ACA itself is still law. The question is increasingly about how generous the government should be in subsidizing it. And there are essentially three broad policy possibilities:

For one, Congress could pass a law restoring the larger subsidies, particularly to those with income 400% above the federal baseline. This would make the Marketplace insurance cheaper for many people again, but it would also cost the federal government more money. For example, the Congressional Budget Office estimated that permanently extending the enhanced premium-tax-credit structure would increase federal

deficits by roughly $350 billion over 2026-35 while increasing the number of insured people by about 3.8 million in 2035 (CBO, 2025).

They could keep the ACA under the original subsidy structure which is essentially the status quo in 2026. The ACA Marketplace continues operating, but people receive the less generous original subsidies rather than the enhanced 2021-25 version. That means the federal government spends less, but some people pay more for insurance or potentially leave the Marketplace.

Or, Congress could also negotiate a middle ground, and change the ACA subsidies…again.

The expiration of the enhanced Premium Tax Credits does not mean the Affordable Care Act is disappearing. Rather, it marks a shift back towards the original ACA structure, pre COVID-19, which raises a larger question about the future of health-care affordability in the United States. The enhanced credits helped expand access to Marketplace coverage, but maintaining those subsidies also requires substantial federal spending. Their expiration has already contributed to higher out-of-pocket premiums for many enrollees and declining Marketplace enrollment, making the consequent change in policy increasingly visible.

Moving forward, Congress will have to determine whether the original subsidy structure is sufficient, whether the enhanced credits should be restored, or whether another approach should be adopted. The future of the ACA is not simply a question of whether the law survives, but of how policymakers choose to balance these competing priorities.

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